

HOW TO OPEN A BRAIN INJURY ADULT RESIDENTIAL FACILITY IN CALIFORNIA

A California-licensed Adult Residential Facility (ARF) serving adults with brain injury — non-medical residential care, supervision, assistance with self-administered medication, and support toward greater independence.

A step-by-step startup and operations guide

Licensing (CDSS) · Staff & training · Funding · The program · Accreditation

Regulatory basis: California Code of Regulations, Title 22, Division 6, Chapter 6 (Adult Residential Facilities), and the California Community Care Facilities Act (Health and Safety Code Section 1500 et seq.); licensed by the California Department of Social Services (CDSS), Community Care Licensing Division. Funding: SSI/SSP, Regional Center, CalAIM Community Supports, and private pay.

How to Use This Guide

This guide walks you through opening and operating an Adult Residential Facility (ARF) in California that serves adults with brain injury. An ARF is California's residential-care license for adults ages 18 to 59 who have a physical, developmental, or mental disability — licensed by the California Department of Social Services (CDSS), Community Care Licensing Division, under Title 22 and the Community Care Facilities Act. There is one licensing path (get the home licensed as an ARF), and a funding picture that is genuinely a patchwork — which this guide walks through honestly.

Important — two things to understand up front

First, an ARF is a **non-medical** facility: staff *assist* residents with self-administered medication and store medications centrally — they do not *administer* medications as a nursing act, and the ARF provides no skilled nursing (that comes from a physician, licensed professional, or home health). Second, this guide is an educational resource, not legal, financial, tax, or billing advice — verify every step with CDSS and, for funding, with the resident's payer, before you act.

The big picture, in one paragraph

Here is the whole business in a nutshell. You license a home with the State (CDSS) as an Adult Residential Facility built for adults with brain injury. You staff it with cleared, trained people, and run it on written policies and each resident's Needs and Services Plan. You fill it through relationships with rehabilitation hospitals, regional centers, Medi-Cal managed-care plans, and families. And you fund it through a blend — SSI/SSP board-and-care, Regional Center placements, CalAIM Community Supports, county augmentations, and private pay — because no single source pays enough on its own. License it, staff it, fill it, fund it.

The path at a glance — step by step

1. Set up the business — form your California entity, secure the home, and confirm the address works (for six or fewer residents, an ARF is a residential use by right; see Part 3).
2. Complete CDSS orientation — register for and attend the Adult Residential Facility Component I Orientation.
3. Get cleared — Live Scan criminal record clearance for you and anyone present in the facility (H.S.C. § 1522).
4. Certify as an administrator — the 35-hour ARF Administrator Certification and the Department exam.
5. Prepare the home — fire clearance, and confirm the address is not overconcentrated (within 300 feet of another facility).
6. Write your policies — your Title 22 policy manual and the forms it references.
7. Apply to CDSS — submit the ARF application with the required documents and fee; complete the Component II Orientation with your analyst.
8. Pass the licensing inspection — CDSS issues your license once you demonstrate compliance.
9. Line up funding and referrals — SSI/SSP, regional-center vendorization, CalAIM contracting, and referral relationships.
10. Admit and document — complete assessments and Needs and Services Plans, deliver services, and document to stay inspection-ready.

Part 1 — What This Business Is

A California Adult Residential Facility provides 24-hour non-medical care and supervision to adults 18 to 59 who have a physical, developmental, or mental disability — room, board, personal-care assistance, supervision, assistance with self-administered medication, central storage of medications, and arrangement of medical and dental care. It does not provide skilled nursing or administer medications as a nursing act; those come from licensed professionals or home health.

License: Adult Residential Facility, issued by CDSS Community Care Licensing under Title 22, Division 6, Chapter 6, and the Community Care Facilities Act.

Why brain injury is its own niche — and why the ARF is the license

Many brain-injury residents are younger adults — exactly the ARF age range — who were independent before a crash, fall, assault, stroke, or overdose. They may walk and talk normally but struggle with memory, initiation, judgment, impulse control, and fatigue, and they frequently take anticonvulsants and other medications that must be taken accurately and on schedule. A home that provides structure, supervision, and reliable support with those medications, plus skill-building, is genuinely hard to find — which is why hospitals, regional centers, and families are looking for you. For adults with brain injury, the ARF is the license: the RCFE serves people 60 and older, and the ARF for Persons with Special Health Care Needs requires regional-center clients with developmental disabilities.

What you are actually selling is trust and specialization: a safe, structured home where a person with a brain injury can rebuild daily-living skills and, where possible, move toward greater independence. The demand is real and under-served — California has a well-documented shortage of board-and-care beds — so a well-run specialized home enters a market that needs it.

It helps to be clear-eyed about who your residents are. Some are regional-center consumers whose brain injury dates to childhood; some are adults injured in a crash or a fall who are stepping down from a rehabilitation hospital or a nursing facility; some are private-pay families who simply cannot find a home equipped for their loved one. Each comes with a different funding path (Part 6) and a different set of needs, but all need the same core things you provide: a safe, structured, non-medical home with trained staff who understand brain injury. Building your admissions around that mix — rather than hoping a single funder will fill every bed — is what makes the business work.

Part 2 — Who Regulates You

- **CDSS, Community Care Licensing Division (CCLD)** — licenses your ARF, sets the Title 22 standards, and conducts licensing and complaint inspections through Licensing Program Analysts.
- **Regional centers (through DDS)** — place and fund residents who are regional-center consumers, and vendorize (approve and rate) the facilities that serve them.
- **Medi-Cal managed-care plans / DHCS** — administer CalAIM Community Supports, which can pay the care portion for members moving out of or avoiding a nursing facility.
- **Long-Term Care Ombudsman & Adult Protective Services** — resident advocacy and the investigation of abuse, neglect, and exploitation.

Keep the sides straight, because it saves weeks of being bounced around: CDSS is your licensing regulator (it decides whether you can open and inspects you); regional centers and Medi-Cal plans are payer/placement bodies (being licensed does not make you a regional-center vendor or a CalAIM provider, and vice versa); and the Ombudsman and APS are oversight bodies around resident rights and safety. You deal with each through a different office — which is why the contact list near the end of this guide is organized by what you need.

One more distinction worth internalizing early: licensing and funding are entirely separate tracks that run on their own timelines. You can hold a valid ARF license and still have no regional-center vendorization or CalAIM contract in place, and you can be deep into funding conversations while your license application is still pending. Sequence them deliberately — you generally need the license (or a clear path to it) before a regional center will vendorize you or a Medi-Cal plan will contract with you — and start the funding conversations early, because they are slower than operators expect.

Part 3 — Business, Property, and the California Zoning Advantage

- Form your California entity (usually an LLC or corporation) with the Secretary of State, get an EIN, and open a business bank account — keep business and personal funds strictly separate.

- Secure the home (own, lease, or long-term rental); the address goes on your application.
- Get liability, professional (abuse-and-molestation), property, and workers'-compensation insurance appropriate to a licensed ARF — tell the insurer plainly you serve adults with brain injury.
- Obtain a National Provider Identifier (NPI) if you will contract with Medi-Cal managed-care plans for CalAIM.

What each of these really means

The entity is your legal shield: form an LLC or corporation so the business — not you personally — holds the license, signs the lease, and carries the liability, and keep the business's money strictly separate from your own, because commingling is one of the fastest ways to lose that protection. CDSS will also want to see you have the financial resources to run the home (H.S.C. § 1520), so line up your financing before you apply. Insurance is not one policy: plan for general liability, professional (or 'abuse and molestation') coverage, property, workers' compensation once you have employees, and commercial auto if you transport residents — and tell the insurer plainly that you serve adults with brain injury in a licensed ARF, because underpricing the risk by describing it as ordinary housing leaves you uncovered when it matters most.

The California zoning advantage — six or fewer residents

Under Health & Safety Code § 1566.3, a licensed residential facility that serves **six or fewer residents** is a residential use of property, and the residents and operator count as a 'family.' No conditional use permit, zoning variance, or other zoning clearance can be required that isn't required of any other family home in that zone — so a small ARF can generally open in any residential neighborhood by right. ('Six or fewer' does not count you, your family, or staff.) You must still meet neutral building and health-and-safety codes, obtain a fire clearance, and satisfy the 300-foot overconcentration rule — and a facility of seven or more generally does need a conditional use permit.

This is a genuine advantage over most states, where opening a group home means a zoning fight. In California, a six-bed ARF is treated like a family residence. Confirm your address with the local planning department before you commit, and check the overconcentration rule.

Two cautions keep this advantage from becoming a trap. First, overconcentration: even a six-or-fewer home can be blocked if it sits within 300 feet of another licensed community care facility (H.S.C. § 1520.7), so check the CDSS care-facility search for your block before you sign anything. Second, 'residential use' protects you from special zoning — it does not exempt you from neutral building codes, a fire clearance, or occupancy limits that apply to any home; and it applies only to a licensed facility, so the protection attaches once you hold (or are properly pursuing) your ARF license. Get the planning department's confirmation in writing, and bring the fire authority in early, because fire-clearance requirements can shape your floor plan.

Part 4 — Get Licensed: The CDSS Process

1. Register for and complete the ARF Component I Orientation (an overview of the program, laws, facility types, and the application).
2. Complete Live Scan fingerprinting and obtain criminal record clearance (or an approved exemption) for yourself and anyone who will be present in the facility (H.S.C. § 1522).
3. Complete the 35-hour Adult Residential Facility Administrator Certification and pass the Department-administered exam.
4. Obtain your fire clearance and confirm the address is not overconcentrated (within 300 feet of another community care facility).
5. Submit the ARF application with the required forms, documents, and fee, demonstrating the financial resources to maintain the required level of service (H.S.C. § 1520).
6. Complete the Component II Orientation one-on-one with your assigned licensing analyst.
7. Pass the licensing inspection; CDSS issues your license once you demonstrate compliance with Title 22.

The documents and steps you'll assemble

Give yourself time to line these up — several depend on outside parties (fingerprint clearance, fire authority, a CPA or bank for the financial showing) and can't be rushed at the end:

- ☐ Component I Orientation certificate (and Component II completed with your analyst).
- ☐ Live Scan criminal record clearances for you and everyone present in the facility.
- ☐ Your 35-hour ARF Administrator Certificate and passing exam result.
- ☐ The completed CDSS application and fee, with the financial showing (H.S.C. § 1520).
- ☐ Fire clearance for your capacity and residents' ambulatory status.
- ☐ Overconcentration verification (not within 300 feet of another facility).
- ☐ Your Title 22 policies and procedures, and the resident forms they reference.

Where your manual and forms fit. Your Policy & Procedure Manual and Forms Packet are built to the Title 22 ARF requirements — have them ready before you apply, since your policies and the resident forms are part of demonstrating you can operate in compliance.

What the inspection looks at — and where homes lose points

Expect the analyst to tour the home, review your policies and personnel files (clearance, health screening, training), check the physical plant and fire clearance, and confirm your admission, assessment, Needs and Services Plan, medication-assistance, and record systems match your policies. The deficiencies that most often delay a first license are small and avoidable: missing Live Scan clearances, undocumented staff training, an incomplete Needs and Services Plan, medications not stored or recorded correctly, and gaps in required postings (personal rights, the complaint number). Walk the home with an inspector's eyes a week ahead and fix the small things.

The inspection is not a pop quiz — it is a check that the home you described on paper actually exists and runs the way your manual promises. If the analyst finds deficiencies, you are typically given the chance to correct them rather than being denied outright; take the correction seriously, fix the root cause, and document it. Plan for months, not weeks, from starting the process to holding your license: orientation, Live Scan clearances, and the 35-hour administrator certification all take time, and the application review and inspection add more. Budget a runway for a licensed-but-empty home, because funding (regional-center vendorization and CalAIM contracts especially) is rarely in place the day you open.

Part 5 — Staff, Training, and the Care Team

- **Administrator:** a certified ARF administrator (35-hour certification plus the Department exam) manages the facility, with a qualified person designated to cover any absence.
- **Criminal record clearance:** everyone present in the facility clears through Live Scan (DOJ/FBI) before contact with residents; California's Rap Back continuously monitors for new arrests.
- **Training (§ 80065):** staff are trained in the population's needs, food handling, resident care and communication, assisting with self-administered medications, recognizing early signs of illness, and community resources — plus brain-injury, seizure-response, and de-escalation content.
- **Staffing:** enough staff to meet residents' needs at all times with 24-hour coverage; for regional-center consumers, at least one direct-care staff per three residents.

Staffing is your biggest cost and your biggest challenge, and brain injury raises the bar: these residents need more supervision and more consistency than a typical census, so plan coverage around real needs and invest in retention — a rotating cast of unfamiliar faces destabilizes someone with memory and processing deficits. Think of yourself as running a care team: the resident's therapists, physicians, behavioral providers, and regional-center or CalAIM care manager are all part of it, and your job is to coordinate them around each resident's Needs and Services Plan and communicate changes quickly.

The medication reality, stated plainly

An ARF assists with self-administered medication and stores medications centrally — reminding, observing, and making medications available — but does not administer them as a nursing act. For a brain-injury population on anticonvulsants and psychotropics, this is workable but must be planned: identify each resident's medication-assistance level in the Needs and Services Plan, keep the current-medication record accurate, and where a resident cannot safely manage medications even with assistance, arrange a licensed professional or home health — or recognize the resident needs a higher level of care.

Supervision and safety

Brain injury drives your supervision model. Residents may have impaired safety awareness, impulsivity, and elopement risk, so you set each resident's supervision level from their assessment rather than a one-size rule, and you provide awake overnight coverage where the census and needs require it. California lets an approved facility use secured perimeters or delayed egress only under Health & Safety Code § 1531.15, with added staff training — otherwise you manage risk with the least restriction that keeps residents safe: environmental cues, redirection, structured activity, and monitoring, while honoring the personal right of a capable resident to come and go. Document each resident's hazard-awareness and elopement precautions in the Needs and Services Plan, and keep a current photo and a written response protocol for anyone at risk of leaving unsafely.

Building a team that stays

Staffing decides whether your program is good or merely licensed, so treat retention as a core operating priority, not an afterthought. Turnover is expensive twice over — in recruiting and training cost, and in the setback to residents who lose the familiar staff who knew their cues. Pay fairly, train genuinely (beyond the Title 22 minimum, into brain-injury care, seizure response, and de-escalation), schedule for continuity so residents see the same faces, validate competency before staff work unsupervised, and build a culture where staff feel respected and supported. Every direct-care person must clear Live Scan before contact with residents and be trained — including in assisting with self-administered medication — before doing that work. A small, stable, well-trained team beats a larger, churning one every time, and it is the single thing families and referral sources notice first.

Part 6 — How You Get Paid — Funding the Home

Here is the honest funding picture, and it is the part of California that surprises people. There is no single clean Medicaid per-diem for an ARF. You build funding from these sources:

Source	What it pays / how	The reality
SSI/SSP Non-Medical Out-of-Home Care	The board-and-care base for ARF residents — a state-set monthly rate (\$1,626.07 for an individual in 2026, of which the facility receives about \$1,444.07; the resident keeps a small personal-needs allowance). You must accept this government rate as payment in full for an SSI/SSP resident.	It is low — which is exactly why California has a 'board-and-care crisis' and few operators take SSI-only residents. Best combined with other sources.
Regional Center	Tiered placement rates — far higher than SSI — for residents who are regional center consumers. You become a vendored provider through your area's regional center.	Only for residents with a developmental disability — for brain injury, generally an injury before age 18. Adult-onset TBI usually does not qualify.
CalAIM Community Supports	A Medi-Cal managed-care plan pays the 'assisted living' (care) portion in your ARF for a member transitioning out of, or diverting from, a nursing facility; the member pays room and board from SSI.	The newer, growing path. You contract with the member's Medi-Cal managed-care plan; eligibility runs through the plan's CalAIM referral process.
County augmentations ('patches')	Some counties add a supplemental daily rate on top of	Mostly for serious mental illness; varies by

Source	What it pays / how	The reality
	SSI for 'augmented' board and care, often using Mental Health Services Act funds.	county; reported roughly \$15–\$225/day.
Private pay	Resident or family pays for care and room and board under the Admission Agreement.	Commonly \$2,500–\$4,000+/month, higher in the Bay Area and Los Angeles.

The honest headline

SSI/SSP is the base, but it is low. Regional Center pays well but only for residents whose disability is developmental (for brain injury, usually an injury before age 18). CalAIM Community Supports is the promising newer Medi-Cal path for residents leaving or avoiding a nursing facility. Most successful homes blend sources and treat private pay as a deliberate base, not a fallback.

A word on who qualifies for what, because it drives your whole census strategy. Almost any low-income resident can bring SSI/SSP board-and-care, but that rate alone rarely covers the cost of specialized brain-injury care. Regional Center pays far better — but only for residents whose brain injury counts as a developmental disability, which generally means it occurred before age 18 and causes substantial disability; adult-onset TBI usually does not qualify, and this catches many operators by surprise. CalAIM Community Supports is the path that has opened up for adult-onset residents: if a resident is leaving a nursing facility (or would otherwise need one) and has Medi-Cal managed care, the plan can pay the care portion while the resident pays room and board. Knowing which bucket each prospective resident falls into — before you admit — is how you keep the home financially viable.

Regional-center vendorization and CalAIM contracting

To serve regional-center consumers, you go through vendorization with the regional center serving your county — which sets a placement rate based on the level of service. To serve CalAIM members, you contract with the Medi-Cal managed-care plans in your area and work their Community Supports referral process (the member is authorized by the plan, pays room and board from SSI, and the plan pays the care portion). Both take time to set up, so start early, and don't count on either being in place the day you open.

Pricing your private-pay rate

Price private pay deliberately from your real costs — the home, staffing (your largest cost, and higher for brain injury because of the supervision required), food, utilities, insurance, training, and a margin — and put the rate, what it includes, and your refund and rate-change terms clearly in the Admission Agreement. Remember that for an SSI/SSP resident you may charge only the government rate; price and plan your census accordingly.

The other money

- **Veterans, auto, and workers' comp:** because many brain injuries are accident- or service-related, a resident may have VA benefits, auto-insurance, or workers'-compensation coverage — always ask.
- **The state TBI Program:** the California Department of Rehabilitation funds community-based TBI services at designated provider sites — a services-and-referral resource rather than a per-bed funder, but worth knowing.

The practical art of funding an ARF is stacking these sources across your beds. A viable home might run a few private-pay residents whose rates carry the fixed costs, a couple of regional-center placements at solid rates, one or two CalAIM residents whose plans pay the care portion, and the balance in SSI residents it can serve within the government rate — with the exact mix driven by your local market and the residents you can safely serve. Model your census this way from the start, and revisit it as rates change; a home built on the assumption that one funder will fill every bed is the home that struggles.

Part 7 — The Program and Accreditation

Beyond housing and supervision, a strong brain-injury ARF assesses each resident, builds an individualized Needs and Services Plan with community-integration goals, uses brain-injury-informed cueing and behavioral supports, assists reliably with medications, and plans community re-entry — tracking outcomes over time.

In practice that means a real assessment at admission of each resident's cognitive, physical, behavioral, and daily-living abilities, refreshed when things change; a plan with concrete, measurable goals (managing a medication routine with fewer prompts, using community transportation) developed with the resident and coordinated with their care manager; a predictable daily structure, because structure is medicine for a brain injury; and positive, least-restrictive behavioral supports rather than confrontation. Community integration — supervised, graduated practice in real settings — is what builds the skills a resident needs to move toward greater independence.

The interdisciplinary approach is what turns a residence into a rehabilitation-oriented program: rather than each provider working in isolation, an interdisciplinary team — the resident and representative, physician and specialists, therapists, the regional-center or CalAIM care manager, home health where involved, and trained direct-care staff — aligns the resident's therapy goals, behavioral supports, medication routine, and daily structure so they reinforce one another. Treating the program as transitional, aimed at moving residents toward the most independent setting they can achieve rather than indefinite custodial care, is central both to good brain-injury care and to how payers, regional centers, and CalAIM plans evaluate you — and it is where tracking outcomes (functional gains, length of stay, where residents go when they leave) turns your program into something you can prove.

Accreditation — a market advantage. Payers, regional centers, and referral sources often prefer national rehabilitation accreditation (the Joint Commission or CARF, which specializes in brain-injury programs) for brain-injury providers. Building your program to align with these frameworks — without reproducing their proprietary standards — strengthens contracting and inspection readiness, and your outcome data becomes the evidence that makes the case. Your Policy & Procedure Manual is structured to support this alignment.

What a good day in the home looks like

It helps to picture the program concretely. A good day is structured and predictable: residents wake and follow a routine supported by cues and checklists; medications are made available and taken with the assistance each resident needs, logged accurately; staff who know each resident run the morning by their strategies, not by a generic schedule. Through the day there is real activity — skill-building, supervised community outings, therapy appointments the home has coordinated — chosen for engagement and progress rather than to fill time. Behavior is handled with redirection and de-escalation, never punishment, because episodes are understood as symptoms of the injury. And everything is documented as it happens, so the Needs and Services Plan stays current and the next shift picks up seamlessly. That consistency — the same routine, delivered the same way, by familiar staff — is not a nicety; for a person with a brain injury it is the intervention, and it is what referral sources and families can feel the moment they walk in.

Part 8 — Timeline, Costs, and Two Worked Examples

Rough sequence

Plan for several months from start to first resident: form the entity and secure the home → complete orientation, Live Scan, and administrator certification → prepare the home and get fire clearance → submit the application and complete Component II → pass the licensing inspection → receive your license → set up funding (SSI/SSP, regional-center vendorization, CalAIM contracts) and referrals → admit. Verify current timeframes with CDSS.

Typical startup costs to budget

- The home (lease or purchase), accessibility modifications, furnishings, and fire-safety features.
- Live Scan, administrator certification, and staff training.
- The CDSS application fee and fire-clearance costs.

- Insurance, business setup, and your Policy & Procedure Manual and Forms Packet.
- A cash reserve — the home may run partly empty and unfunded for a while (SSI is low, and regional-center/CalAIM setup takes time); undercapitalization, not lack of demand, is what closes new homes.

Worked example A — a six-bed ARF in the Los Angeles area

A single-family home is licensed as a six-bed ARF serving adults with brain injury in the Los Angeles area. Because it serves six or fewer residents, it opens in a residential neighborhood as a by-right residential use (§ 1566.3) — no conditional use permit — after building permits, fire clearance, and an overconcentration check. It takes referrals from area rehabilitation and trauma programs (Rancho Los Amigos National Rehabilitation Center is a major Southern California brain-injury source) and from regional-center and Medi-Cal-plan care managers. Its funding is a mix: SSI/SSP board-and-care for some residents, a regional-center placement rate for a resident whose injury occurred in childhood, a CalAIM Community Supports authorization for a resident who transitioned out of a nursing facility, and private pay for others. Confirm current rates before projecting revenue.

Worked example B — an ARF in the San Francisco Bay Area

An ARF in the Bay Area runs the same model. It draws referrals from area rehabilitation programs and trauma centers and from the regional center and Medi-Cal plans serving the city. The licensing, staffing, medication, and funding steps are identical to the Los Angeles home; the differences are the local permitting offices below and the higher private-pay rates the Bay Area market supports. Given how low SSI is, it leans on regional-center, CalAIM, and private-pay residents while filling with SSI residents it can serve within the government rate.

The economics differ by market in a way worth planning around. In high-cost areas like the Bay Area and coastal Los Angeles, real estate and wages are steep, but private-pay rates are correspondingly higher and demand is intense — so a smaller, higher-acuity, largely private-pay or regional-center home can pencil out. In lower-cost inland areas, private-pay rates are lower but so are your costs, and SSI/CalAIM residents make up more of the census. Neither is inherently better; what matters is matching your cost structure, your capacity, and your funding mix to your local market, and pricing your private-pay rate from your actual costs rather than a number you hope the market will bear.

In both markets, discharge planners at rehabilitation hospitals and trauma centers, regional-center service coordinators, and Medi-Cal-plan care managers are your primary referral sources, alongside families seeking specialized brain-injury care. Actual revenue depends on each resident's funding and your private-pay rate; confirm current figures with CDSS, the regional center, and the resident's payer.

Getting your first residents

Referrals come from relationships, and they take time to build — start before you have an empty bed. Introduce yourself and your program to the discharge-planning teams at your area's rehabilitation hospitals and trauma centers, to the service coordinators at your regional center, and to the care managers at the Medi-Cal managed-care plans in your county. Be specific about what you offer that ordinary board-and-care homes do not: brain-injury-trained staff, structure and supervision, reliable medication assistance, and community-integration goals. Families searching for specialized care are a second channel — an honest description of your program and admission process helps them find and choose you. Keep your outcome data from the first resident forward, because it becomes the proof of value that referral sources, regional centers, and plans respond to.

Part 9 — Common Mistakes That Delay or Sink an ARF

Most homes that stumble don't fail on the big things — they fail on avoidable ones. Learn these before they cost you.

- **Expecting a clean Medicaid per-diem.** There isn't one for an ARF. Build your model on the SSI/regional-center/CalAIM/private-pay blend from the start.

- **Assuming staff can administer medications.** An ARF is non-medical — assistance and central storage only. Plan licensed support for residents who need more.
- **Under-budgeting the pre-revenue runway.** You pay for the home, staff, and setup for months — and regional-center vendorization and CalAIM contracts take time to finalize.
- **Missing the overconcentration rule.** Even with the six-or-fewer zoning protection, a facility within 300 feet of another community care facility can be blocked. Check first.
- **Thin clearances and files.** Missing Live Scan clearances, undocumented training, and incomplete Needs and Services Plans are the deficiencies that most often delay a first license.
- **Admitting residents you can't serve.** A resident with a prohibited health condition, or who needs skilled nursing beyond what home health can support, exceeds ARF authority. Assess honestly.
- **Weak abuse-reporting discipline.** As a mandated reporter, call immediately and file the written report within two working days — never delay for internal review.

Part 10 — Your First 90 Days Operating

Getting the license is the start line, not the finish. Your first three months set whether the home runs smoothly.

- **Document everything from day one.** Assessments, Needs and Services Plans, medication-assistance records, unusual-incident reports, drills, and training — your first inspection looks at what you actually did.
- **Finish your funding setup.** Complete regional-center vendorization and CalAIM contracting so funding can flow, and confirm SSI arrangements for your SSI residents.
- **Lock in referral relationships.** Introduce yourself to rehab-hospital discharge planners, regional-center coordinators, and Medi-Cal-plan care managers — referrals come from relationships and take time to build.
- **Build your staff into a team.** Consistency across shifts is everything for brain-injury residents — keep each resident's strategies visible and address turnover fast.
- **Run your medication system tightly.** Reliable medication assistance and accurate records are your core clinical promise — audit them from the start.
- **Track outcomes from your first resident.** Outcome data becomes your proof of value for payers, regional centers, accreditation, and referral sources.

Above all, treat your first ninety days as the period that sets your reputation. Regional-center coordinators, plan care managers, and hospital discharge planners talk to each other, and a home that is organized, responsive, and honest about what it can and cannot serve earns the referrals that keep it full. Get your documentation habits right now, while the census is small, and they will hold as you grow.

Part 11 — When They Say “Check Locally”: Who to Call & What to Ask

Some things are handled by the State (CDSS, regional centers, Medi-Cal plans), and some by your local city or county (building permits, fire clearance, zoning confirmation). Here is who to call and exactly what to ask. Verify current numbers before you rely on them.

The single most useful habit here is to get answers in writing. A planning department's confirmation that your six-or-fewer ARF is a by-right residential use, a fire authority's statement of what clearance you need, an overconcentration check for your block — each of these can be an email you keep in your file, and each protects you if a question comes up later. Ask for the person's name and a written confirmation every time, and start these calls early, because fire and building requirements can shape decisions you'd rather not revisit after you've signed a lease.

The statewide offices (same for everyone)

For...	Who to contact
Licensing your ARF (application, standards, inspection, complaints)	CDSS Community Care Licensing — 844-538-8766 (844-LET US NO) · letusno@dss.ca.gov · complaints.cclcd.dss.ca.gov
Regional-center funding & vendorization	The regional center serving your county — find it through the Department of Developmental Services (dds.ca.gov)
CalAIM Community Supports	The member's Medi-Cal managed-care plan (Community Supports / ECM referral process)
State TBI services & referral	California Department of Rehabilitation — Traumatic Brain Injury Program (dor.ca.gov)
Resident advocacy	Long-Term Care Ombudsman — statewide CRISISline 1-800-231-4024
Reporting abuse, neglect, or exploitation	911 if in immediate danger · your county Adult Protective Services (e.g., Los Angeles County APS 877-477-3646) · CDSS 844-538-8766 · report form SOC 341
Verify a facility / check overconcentration	CDSS Care Facility Search — cclcd.dss.ca.gov/carefacilitysearch

The local calls — and exactly what to ask

① City or County Planning — “Confirm my address for a six-or-fewer ARF.”

- “I’m opening a licensed Adult Residential Facility for six or fewer adults with disabilities. Under Health & Safety Code § 1566.3 that’s a residential use by right — can you confirm there’s no conditional use permit required at [address]?”
- “Are there any building-height, setback, or sign rules that apply the same as to other homes here?”
- “Is there another licensed community care facility within 300 feet of this address?” (overconcentration).

② Building — confirm what permits any modifications need, and your certificate of occupancy.

③ Fire — “What fire clearance does an ARF of [capacity], with these residents’ ambulatory status, need — and can you schedule the inspection I’ll need for my CDSS license?”

Your two-metro call list — fully worked

LOS ANGELES

- **Building permits:** City of L.A. Department of Building and Safety (LADBS), 201 N. Figueroa St. — 213-473-3231 or 3-1-1; apply online through PermitLA / ePlanLA; ladbs.org.
- **Zoning confirmation:** Los Angeles City Planning (planning.lacity.gov) — use ZIMAS to check your parcel’s zoning.
- **If your address is in unincorporated county:** L.A. County Regional Planning, 213-974-6411 — and county fire for the clearance.
- **Fire:** LAFD (city) or L.A. County Fire (its Health/Schools & Institutions unit inspects residential care facilities in county areas) for your fire clearance.
- **The local quirk:** greater L.A. is a patchwork of the City of L.A. and dozens of independent cities plus unincorporated county — confirm which jurisdiction your exact address is in before you call.
- **Referrals:** Rancho Los Amigos National Rehabilitation Center and area trauma centers; the regional center serving your area.

SAN FRANCISCO BAY AREA

- **One-stop Permit Center:** the San Francisco Permit Center (49 South Van Ness Ave) combines Planning, the Department of Building Inspection, and the Fire Department; email PermitSF@sfgov.org.
- **Zoning confirmation:** SF Planning — use the Property Information Map (sfplanninggis.org/pim) to check a parcel’s zoning, or request a Zoning Verification Letter.

- **Building permits:** SF Department of Building Inspection (sfdbi.org), the lead agency for building permits, with electronic plan review.
- **The local quirk:** San Francisco is dense and expensive with seismic and slope overlays; the six-or-fewer residential-use protection still applies, but building, fire, and the 300-foot overconcentration rule matter — check the Property Information Map early.
- **Referrals:** area rehabilitation and trauma programs; the regional center serving the city; Medi-Cal managed-care plan care managers.

How to find your own city or county's numbers (if you're elsewhere)

1. Confirm your jurisdiction: search “[your city] CA planning department” and “[your county] CA building and safety.” Many California addresses are in a city; some are in unincorporated county.
2. Ask Planning the six-or-fewer question above and get the answer in writing.
3. Ask the Fire authority what clearance your ARF needs for the CDSS license.
4. For funding, find your regional center through dds.ca.gov and identify the Medi-Cal managed-care plans in your county for CalAIM.
5. Not sure who covers you? Call your county's main line and ask for planning/building.

Key Terms at a Glance

The alphabet soup you'll hear, in plain language:

- **ARF (Adult Residential Facility):** California's non-medical residential-care license for adults 18–59 with disabilities — the fit for brain injury.
- **RCFE:** Residential Care Facility for the Elderly — the 60-and-older version; not the fit for younger TBI adults.
- **CDSS / CCLD:** the Department of Social Services and its Community Care Licensing Division — licenses and inspects your ARF.
- **Needs and Services Plan:** the individualized plan (§ 80068.2 / 85068.2) that drives each resident's care.
- **SSI/SSP NMOHC:** the Non-Medical Out-of-Home Care rate — the board-and-care base payment for ARF residents on SSI.
- **Regional Center:** the DDS-funded agency that places and funds residents with developmental disabilities.
- **CalAIM Community Supports:** Medi-Cal managed-care services that can pay the care portion for nursing-facility diversion/transition.
- **Live Scan / Rap Back:** California's electronic fingerprint clearance and continuous-monitoring system (H.S.C. § 1522).
- **Overconcentration:** the 300-foot separation rule between community care facilities (H.S.C. § 1520.7).
- **CARF / Joint Commission:** the two national accreditors payers recognize; CARF specializes in brain-injury programs.

Core Rules & References

- **Title 22, Division 6, Chapter 6** — Adult Residential Facilities (the licensing regulations, including the 80000- and 85000-series sections).
- **Community Care Facilities Act** — Health & Safety Code § 1500 et seq.; application (§ 1520); criminal record clearance (§ 1522); overconcentration (§ 1520.7); six-or-fewer residential use (§ 1566.3).
- **Abuse reporting** — Elder and Dependent Adult Civil Protection Act, Welfare & Institutions Code § 15600 et seq. (report form SOC 341).
- **Rules & statutes online** — leginfo.ca.gov; licensing information at cdss.ca.gov/inforesources/community-care-licensing.

Your Compliance Foundation

Opening the home is the first milestone; staying compliant with Title 22 — and ready for accreditation — is the ongoing work. Two companion resources are built for that: a Policy & Procedure Manual covering the ARF licensing standards and the accreditation framework, and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone CDSS and an accreditor expect.

Before you rely on any step in this guide

Verify current requirements, forms, fees, and rates with CDSS and, for funding, with your regional center, the Medi-Cal managed-care plans in your county, and the resident's payer. Remember that an ARF is non-medical (assistance with self-administered medication, not administration) and that its funding is a blend, not a single per-diem — confirm each resident's funding before admission.